



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 255/10

BEFORE: M.C. Smith : Vice-Chair

HEARING: January 24, 2011 at Toronto
Oral

DATE OF DECISION: January 26, 2011

NEUTRAL CITATION: 2011 ONWSIAT 203

DECISION UNDER APPEAL: WSIB ARO decision dated November 18, 2008

APPEARANCES:

For the worker: Mr. Leo J. Dillon, Lawyer

For the employer: Not participating

Interpreter: N/A

REASONS

(i) Introduction

- [1] The worker appeals the decision of Appeals Resolution Officer (ARO) Mrs. N. Berman, dated November 18, 2008. The worker was represented by Mr. Leo J. Dillon, a lawyer. The employer did not participate in the hearing. As a preliminary matter, Mr. Dillon indicated that the sole issue which the worker wished to pursue at this hearing was entitlement to a permanent aggravation of her pre-existing, non-compensable right carpal tunnel syndrome (CTS), claimed to be the result of a workplace accident on December 7, 2004. At the present time the worker did not wish to pursue either the suitability of the modified duties offered by the employer on December 10, 2004 or her entitlement to full Loss of Earnings (LOE) benefits subsequent to December 13, 2004. Oral evidence was heard from the worker. Submissions were made by Mr. Dillon.

(ii) The Issue

- [2] The worker is seeking entitlement to a permanent aggravation of her pre-existing, non-compensable right carpal tunnel syndrome (CTS). She is claiming that this aggravation was caused by a workplace accident on December 7, 2004.

(iii) Background

- [3] This now 58-year-old Dietary Aide began working with the accident employer, a nursing home, in November 2004. On December 7, 2004 she slipped on a wet floor in the kitchen. Lost time benefits for a right shoulder strain were allowed from December 8 – 12, 2004. Further benefits were not allowed on the basis that on December 10, 2004 the employer had offered modified duties which were deemed to be within the worker's medical restrictions. The worker is seeking entitlement to benefits for a permanent aggravation of her pre-existing, non-compensable right carpal tunnel syndrome (CTS), which she attributes to the accident on December 7, 2004. This is the issue before me.

(iv) Relevant law and policy

- [4] On January 1, 1998, the *Workplace Safety and Insurance Act*, 1997 (WSIA) took effect and applies in this appeal. Pursuant to section 126 of WSIA, the Appeals Tribunal is required to apply any applicable Board policy when making decisions. The Board has identified certain policies applicable to this appeal and I have considered them as necessary.

(v) The worker's testimony

- [5] At the hearing the worker provided the following testimony:
- She is right-handed.
 - Subsequent to the flare-up of her CTS in 1996 she had a cortisone injection, attended physiotherapy, and did exercises for her wrist. Her symptoms eventually eased.
 - She remained off work for about one year. During that time she worked out of her home doing baking for friends. She subsequently found employment in a bakery, where she had light duties.

- She continued to attend physiotherapy intermittently -- about once a month.
- She began working as a Dietary Aide in the nursing home in November 2004.
- On December 7, 2004 she slipped in a puddle of water caused by a leaking dishwasher at work. She landed on her right side, striking her head and straining her neck. The worker testified that she used her outstretched right hand to break her fall, landing on the palm of her right hand.
- She has had continuous severe pain in the right wrist since the accident. The pain radiates from the right shoulder down the inside of her arm into the crook of her elbow and ultimately into the middle finger of the right hand.
- Prior to the accident her pain had been intermittent, but it is now continuous. She has difficulty holding a phone or an umbrella in her right hand for any length of time.
- By August 2005 her wrist was feeling better, and on the advice of a friend she applied for a position as a letter carrier. As part of the application process she was required to complete a written test, and did very well. The worker was hired as a letter carrier in November 2005. She started working part time and was eventually moved to a full time position. Her job as a letter carrier involved pulling a cart, which she was able to do with her left hand. The worker drove to her route, filled her cart, and delivered the letters. She then returned to her car to refill the cart. When asked why she did not return to her employment with the accident employer, she stated that the job of letter carrier paid a much better salary. She has been on medical leave from her job as a letter carrier since December 2, 2010 due to depression.
- Surgery for right-sided CTS is presently booked for February 10, 2011. The worker testified that she had resisted surgery up to this point because she feared it might make things worse, rather than better. However, the pain is now so persistent that she feels she has no alternative.

(vi) The medical evidence

(a) The evidence concerning the worker's pre-existing CTS

[6]

In December 1996 the worker submitted a claim for bilateral CTS, which she attributed to her duties working in a bakery. She reported that the pain had started in August 1996, and now disabled her from working. The worker described numbness in both hands and arms, and stated that she required surgery on both hands. In a decision dated February 6, 1997, the worker's request for initial entitlement for CTS was denied because the medical information indicated that her severe right CTS condition pre-dated her employment at the bakery. Below I review the medical evidence concerning the worker's CTS:

- On September 16, 1996, Dr. J. Turley, a neurologist, reported that the worker had been diagnosed with bilateral CTS in October 1994. He indicated that she had severe bilateral CTS with Wallerian degeneration in the distal median nerve on the right side. There was no abnormality of sensory, motor, or reflex conduction. The doctor recommended decompression surgery, particularly on the right side.

- On October 9, 1996, Dr. T. Fischer, a neurologist, reported that EMG and NCS results obtained from Dr. Turley on September 16, 1996 confirmed the presence of severe bilateral CTS. Dr. Fischer opined that the worker required surgical decompression.
- On November 14, 1996 Dr. S.M. Klodas, the worker's family physician, reported that the worker was unable to work as a result of her CTS.
- On February 3, 1997 Dr. Klodas reported that the worker had severe bilateral CTS, and that she could not use either hand. He indicated that she would try physiotherapy before having surgery.

(b) The medical evidence related to the accident on December 7, 2004

- On November 10, 2004 Dr. Klodas completed a Functional Abilities Form for Timely Return to Work (FAF) indicating that the worker had injured her right shoulder and right arm when she slipped and fell. He indicated that she could not use her right arm, and could only do work with her left hand. The doctor stated that the worker required these restrictions for approximately two weeks, and that he expected a complete recovery. Dr. Klodas recommended that the worker have physiotherapy.
- On January 10, 2005 the worker attended physiotherapy. On the Physiotherapy Assessment Report the therapist provided the worker's history of injury, noting right-sided injury to the shoulder, arm, neck, wrist and rib cage. The physiotherapist provided a diagnosis of cervical spine strain, right shoulder injury, right elbow pain, right wrist strain, and right rib cage injury. The therapist recommended that the worker avoid doing work that required prolonged positions, carrying, lifting, repetitive movement of the right shoulder and lifting above shoulder level on the right.
- On January 21, 2005 Dr. Klodas completed a FAF indicating that the worker could only work with her left hand, and stating that she had aggravated her pre-existing CTS. He opined that the worker was not ready to return to any kind of work, and did not know whether she would have a complete recovery.
- On February 1, 2005 Dr. Klodas submitted a Physician's Progress Report (Form 26) which provided a medical diagnosis of right shoulder injury and right CTS. He reported that the worker was unable to work with her dysfunctional right hand, and stated that her pre-existing CTS had been aggravated by her recent shoulder injury.
- On March 1, 2005 the worker told the Claims Adjudicator that she would be having surgery in March 2005 but was not sure of the date (Board Memo #8).
- On March 24, 2005 Dr. Klodas reported that the worker was disabled due to CTS.
- On May 31, 2005 Dr. Klodas submitted a Health Professional's Report (Form 8) indicating that the worker had developed right shoulder pain on May 24, 2005 as a result of carrying trays and serving meals. He provided a diagnosis of right shoulder strain and CTS, and stated that the worker was unable to work due to pain in the shoulder and numbness in the hand. In a hand-written note on his prescription pad the doctor stated that the worker had been disabled from work since May 25, 2005.

- In a Health Professional's Report dated July 26, 2005 Dr. Klodas reported that he had referred the worker to Dr. Mullen, a neurosurgeon, for her CTS. He also noted that the worker was suffering from depression.
- On October 19, 2005 the worker attended Dr. K. Prutis, a specialist in Physical Medicine and Rehabilitation, for pain in the right shoulder and neck resulting from a slip and fall on December 7, 2004. The doctor noted that the worker's pain was exacerbated by prolonged sitting and standing. The doctor provided a diagnosis of right shoulder strain and contusion and cervical strain. Dr. Prutis booked an MRI of the cervical spine and the right shoulder, and stated that at the present time the worker could not return to work.

(c) The opinion of the Board Medical Consultant

[7] The medical information on file was reviewed by Dr. Grbac, a senior Board Medical Consultant. On February 21, 2007 (Board Memo #20) Dr. Grbac noted that the worker had severe pre-existing bilateral CTS, and opined that the diagnosis of right CTS was not compatible with the accident history:

Clearly the IW had severe pre-existing bilateral CTS. The clinical entry from 10Jan05, a full month beyond the DOA, did not identify any clinical findings which would have supported that there was an aggravation or exacerbation of the IW's CTS secondary to the compensable fall. One would have expected the onset of symptoms to coincide with or follow fairly soon after the DOA particularly given that a need for surgery was identified not even 2 months after the DOA coupled with the clinical finding of right Thenar muscle wasting on 01Feb05. It seems highly unlikely that the onset of symptoms and clinical findings would have developed and progressed so quickly over the course of one month given that there was no documentation of any findings or symptoms as per the detailed physio assessment of 10Jan05.

Compatibility between the diagnosis of a right CTS and the accident history does not appear to be supported.

(d) Medical Discussion Paper on CTS

[8] In February 2000 a Medical Discussion Paper entitled "Carpal Tunnel Syndrome" was prepared for the Tribunal by Dr. Brent Graham, an orthopaedic surgeon with special expertise in hand surgery. The paper was revised in May 2001. In considering the issue of whether trauma to the wrist area could contribute to CTS, Dr. Graham stated that an injury to the wrist, other than a fracture, "would be distinctly unlikely to cause either direct median nerve [damage or compression] or to predispose the patient to developing symptoms of carpal tunnel syndrome due to compression of the median nerve". In a section of the Paper entitled "Repetitive movements and exacerbation of a pre-existing or intermittently symptomatic state of carpal tunnel syndrome", Dr. Graham stated that CTS is a condition characterized by both exacerbations and remissions, and is not influenced by repetitive movement:

The role of repetitive movements has been alluded to above. The data available on this subject suggests little if any relationship between this type of exposure and carpal tunnel syndrome. The exception would be in instances where the repetitive activity requires both frequent and forceful movements. Guidelines for defining a critical frequency and degree of force can be inferred from these reports.

Where there is a pre-existing diagnosis of carpal tunnel syndrome which is claimed to be exacerbated by a work activity, the same issues in establishing causality pertain as in establishing work-relatedness in general. Carpal tunnel syndrome is known to be a

condition that is characterized by both exacerbations and remissions and so the effect of modified work, absences from work and ergonomic modifications to the workplace are difficult to measure.

(vii) The submission of the worker's representative

[9] In written submissions dated June 6, 2008, Mr. Dillon stated that the accident had not implicated the wrist directly, but that the wrist was indirectly affected by the right shoulder injury. He explained that the worker had not complained about CTS until one month after the compensable accident because the mobility of her upper extremity had initially been restricted by her shoulder injury. He argued that it was not until she regained use of her shoulder that she began to develop more and more pain in the carpal tunnel region. The worker's representative stated:

A shoulder impairment will affect the way the upper extremity is used, and this will in turn have a mechanical impact on the remainder of the upper extremity. This would not immediately begin at the time of injury unless the injury implicated the wrist directly. This has not been asserted at any time. However, the fact of the matter is that the problems increased dramatically within a month following the accident. We are left with essentially two choices, that the timing suggests a contribution, or that the incident is entirely coincidental, an Act of God, so to speak. The latter explanation is not very compelling, and is certainly not a medical theory.

[10] In oral submissions at the hearing, Mr. Dillon argued that the worker had in fact injured her wrist when she fell on December 7, 2004, and that it was this trauma to the wrist that had aggravated her CTS. The worker's representative argued that the November 10, 2004 report from Dr. Klodas noted injuries to the right shoulder and arm, and stated that the arm would include the wrist, even if this were not stated explicitly. He also referred to the January 10, 2005 physiotherapy report, noting that the worker's history of injury included right-sided injury to the shoulder, arm, neck, wrist and rib cage. The physiotherapist provided a diagnosis of cervical spine strain, right shoulder injury, right elbow pain, right wrist strain, and right rib cage injury.

[11] Mr. Dillon noted that prior to the accident the worker had been experiencing pain at several places along the right median nerve – the shoulder, the crook of the elbow and the wrist – and that this pre-existing condition had made the median nerve more vulnerable to any damage from the trauma to her wrist when she fell in December 2004. In support of this argument he made reference to the concept of “double crush” mentioned in the Medical Discussion Paper on CTS:

....in the double crush syndrome it is thought that sub-clinical compression of the median nerve at several points in its course between the spinal cord and the carpal tunnel lowers the threshold for symptomatic compression at level of the carpal tunnel. While this idea fits in with some of what is known about peripheral nerve function, it is a largely unproven hypothesis. Furthermore, it can rarely be shown to be present in clinical cases of carpal tunnel syndrome and should seldom, if ever, play a role in the diagnosis or management of carpal tunnel syndrome.

[12] The worker's representative concluded that the worker's CTS had never returned to the pre-accident state, and that the worker was therefore entitled to benefits for a permanent aggravation of her pre-existing CTS.

(viii) Analysis

[13]

In reviewing the evidence, I am not persuaded that the worker suffered a permanent aggravation of her pre-existing CTS for the following reasons:

- The worker's CTS never settled completely and continued to bother her intermittently subsequent to 1994. At the hearing she testified that ever since the initial flare-up in 1994 she regularly attended a physiotherapist for her wrist, usually about once a month.
- The medical information concerning the worker's CTS was not substantially different after the accident than it was before the accident. In both cases, the doctors described the condition as severe and recommended surgery. In 1996, Drs. Turley and Fisher, who are both neurologists, made the diagnosis of severe bilateral CTS with Wallerian degeneration in the distal median nerve on the right side. They opined that the worker required surgical decompression. Dr. Klodas, the worker's family physician, reported that the worker was unable to work as a result of her CTS. Although Dr. Klodas submitted several reports subsequent to the accident in 2004 stating that the worker's pre-existing CTS had been aggravated by her shoulder injury, in my opinion the medical reports do not provide any evidence that her condition was any worse than it had been prior to the accident. In fact, the October 2005 medical report from Dr. K. Prutis, a specialist in Physical Medicine and Rehabilitation, made no mention at all of the worker's CTS. The doctor provided a diagnosis of right shoulder strain and contusion and cervical strain, and booked an MRI of the cervical spine and the right shoulder.
- Although Mr. Dillon argued that the worker's pre-existing CTS made her more susceptible to the trauma of falling on her wrist in November 2004, referring to the "double crush" syndrome mentioned in the Medical Discussion Paper, I note that Dr. Graham stated that this hypothesis was unproven, and was rarely seen in clinical cases. I note further that Dr. Graham explicitly stated that an injury to the wrist, other than a fracture, "would be distinctly unlikely to cause either direct median nerve [damage or compression] or to predispose the patient to developing symptoms of carpal tunnel syndrome due to compression of the median nerve".
- The worker testified that by August 2005 her condition had improved sufficiently for her to look for employment. She was able to use her dominant right hand to take the written test to be a letter carrier. She has been able to work successfully as a letter carrier since November 2005. The worker laid off work recently due to depression, not due to her wrist. I accept the worker's testimony that she was able to pull her cart with her left hand, but find it unlikely that she could perform all her duties as a letter carrier without some use of her dominant right hand.

[14]

In light of the lack of any medical evidence indicating a permanent worsening of the pre-existing CTS, and the demonstrated ability of the worker to return to full time employment, I find that the fall in November 2004 did not cause a permanent aggravation of the worker's CTS condition. Consequently, entitlement is denied.

DISPOSITION

[15] The appeal is denied.

DATED: January 26, 2011

SIGNED: M.C. Smith